

GREENLEAF HEALTH PORTLAND

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ANNUAL PHYSICAL EXAMINATION -- LABORATORY AND CLINICAL REPORT

Patient: Joanne Lawton

Date of Birth: December 3, 1950

Age: 75

Address: 47 Atlantic Street, Portland, Maine 04101

Phone: (207) 555-0134

Medicare ID: 1EG4-TE5-MK72

Visit Date: January 12, 2026

Provider: Robert Chambers, MD

Visit Type: Annual Wellness Examination

VITAL SIGNS

Blood Pressure (sitting, right arm): 138/82 mmHg

Blood Pressure (repeat, 5 min later): 135/80 mmHg

Heart Rate: 72 bpm, regular

Respiratory Rate: 16 breaths per minute

Temperature: 98.4 degrees F

Height: 5 feet 4 inches

Weight: 161 lbs

BMI: 27.6

Provider note: Weight unchanged from 2025 annual visit (162 lbs). BMI remains in overweight range; no acute concern. Discussed continued dietary modifications in context of diabetes management.

BLOOD PRESSURE HISTORY (past 4 visits)

July 2025 (routine): 137/84 mmHg

January 2025 (annual): 140/86 mmHg

July 2024 (routine): 142/88 mmHg

January 2026 (today): 138/82 mmHg

Trend: Stable with mild improvement. Continue Losartan 50mg and Hydrochlorothiazide 12.5mg unchanged. Home monitoring with personal cuff encouraged.

LABORATORY RESULTS

Specimen collected: January 12, 2026 (fasting, 12 hours)

Laboratory: Coastal Diagnostics, Portland, ME

Reference ranges shown in parentheses

COMPLETE METABOLIC PANEL

Glucose, fasting:	148 mg/dL	(70-99 mg/dL)	HIGH
Sodium:	139 mmol/L	(136-145)	Normal
Potassium:	4.1 mmol/L	(3.5-5.0)	Normal
Chloride:	101 mmol/L	(98-107)	Normal
Carbon dioxide:	25 mmol/L	(22-29)	Normal
BUN (blood urea nitrogen):	18 mg/dL	(7-20)	Normal
Creatinine:	0.88 mg/dL	(0.57-1.00)	Normal
eGFR:	69 mL/min/1.73m ²	(>60)	Normal (borderline; monitor)
Calcium:	9.3 mg/dL	(8.5-10.2)	Normal
Total Protein:	7.2 g/dL	(6.3-8.2)	Normal
Albumin:	4.0 g/dL	(3.5-5.0)	Normal
ALT:	22 U/L	(7-35)	Normal
AST:	19 U/L	(10-40)	Normal
Alkaline Phosphatase:	78 U/L	(44-147)	Normal
Total Bilirubin:	0.7 mg/dL	(0.2-1.2)	Normal

HEMOGLOBIN A1c

A1c Result: 7.4% (Target: below 7.0% per ADA guideline)

Interpretation: Elevated above target. Represents average blood glucose of approximately 166 mg/dL over past 3 months. Previous A1c: 7.1% (January 2025). Increase of 0.3%.

Provider note: Discussed A1c trend with patient in detail. Patient is engaged and motivated. Dietary changes underway. Trial period of 3 months before reassessing pharmacologic options. Next A1c target: within 3 months.

LIPID PANEL (fasting)

Total Cholesterol:	204 mg/dL	(Desirable: below 200)	Borderline high
LDL Cholesterol:	148 mg/dL	(Optimal: below 100; Acceptable: below 130 for moderate risk)	ELEVATED
HDL Cholesterol:	52 mg/dL	(Women: above 50)	Normal
Triglycerides:	148 mg/dL	(Normal: below 150)	Normal (borderline)
LDL/HDL Ratio:	2.85		

Provider note: LDL remains at 148, unchanged from last year. Patient continues to decline statin therapy. Advised on dietary interventions (oats, plant sterols, omega-3 fatty acids, walnuts). Document patient preference. Re-evaluate annually.

COMPLETE BLOOD COUNT

WBC:	6.8 x10 ³ /uL	(4.5-11.0)	Normal
RBC:	4.2 x10 ⁶ /uL	(3.92-5.13)	Normal
Hemoglobin:	12.8 g/dL	(11.5-15.5)	Normal
Hematocrit:	38.4%	(34.9-44.5)	Normal
MCV:	91.4 fL	(80-100)	Normal
Platelets:	218 x10 ³ /uL	(150-400)	Normal

THYROID FUNCTION

TSH:	2.1 mIU/L	(0.4-4.5)	Normal
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VITAMIN D (25-OH)

25-OH Vitamin D:	44 ng/mL	(30-80 optimal)	Normal
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Note: Adequate. Continue Vitamin D3 2000 IU daily as currently prescribed.

PREVENTIVE CARE STATUS

Flu vaccine: Received October 14, 2025 at Grace Episcopal Church clinic
COVID vaccine (updated): Received October 14, 2025 (bivalent, same clinic visit)
Pneumococcal vaccine: PPSV23 received 2021; PCV15 recommended for next visit
Colorectal cancer screening: Colonoscopy 2018, negative; next recommended 2028 or no further if patient declines
Mammography: Last performed 2024 (no current finding); due 2026
Bone density (DEXA): Last 2023 (T-score lumbar -1.3, osteopenia); due 2025 -- SCHEDULE
Dental: Under care of Dr. Sullivan; semi-annual cleanings current

CLINICAL IMPRESSIONS AND PLAN

1. Type 2 Diabetes Mellitus, uncontrolled (A1c 7.4%, up from 7.1%)
 - Continue Metformin 1000mg twice daily and Glipizide 5mg each morning
 - Patient advised to maintain daily fasting glucose log
 - Dietary adjustments reviewed in detail with patient
 - Target A1c below 7.0%; next check within 3 months
 - If A1c has not improved at the 3-month recheck, discussion of basal insulin will be initiated
2. Hypertension, stable
 - Continue Losartan 50mg and Hydrochlorothiazide 12.5mg each morning
 - Home monitoring ongoing; patient uses family cuff
 - Target below 130/80 mmHg; current readings acceptable
3. Hyperlipidemia (diet-managed)
 - LDL 148; patient declines statins; dietary counseling provided
 - Recommend increasing omega-3 foods and soluble fiber
4. Osteoarthritis, hands and lower back
 - Acetaminophen as needed; continue heating pad
 - Patient is considering acupuncture; supportive of trial
5. Cataracts, bilateral, mild
 - Under care of Dr. Patterson, Kestrel Healthcare
 - No change to plan at this time
6. eGFR borderline (69)
 - Monitor annually; no dietary protein restriction at this level
 - Continue adequate hydration
7. Preventive
 - DEXA scan overdue; order placed today for scheduling
 - Mammography due 2026; order placed

Follow-up: approximately 3 months post-visit for A1c recheck and medication review.

Robert Chambers, MD
Greenleaf Health Portland
ME License: ME-MD-44217
Date: January 12, 2026